



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : خديجه عبدالوهاب عبدالله هيكل

العنوان : -----

المحافظة : الجيزة الحي / المدينة : البدرشين

تاريخ تسجيل الإستمارة : 23-09-2026

رقم البطاقة : 28601052102269 رقم التليفون : 01013496180

العمر (تاريخ الميلاد) : 05-01-1986 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: ----- التوقيع: -----

الاسم: د/ندى حسن الوظيفة: ----- التوقيع: -----

الاسم: د/نرمين رمضان الوظيفة: ----- التوقيع: -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----

خطة العلاج : -----

المستشفى : ----- قسم : -----

جهة العلاج : ----- نفقات : -----

ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: خديجه عبدالوهاب عبدالله هيكلا National ID: 28601052102269	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة هوية شخصية

خديجة
عبد الوهاب عبدالله هيكل
قرية ابورجوان قبلى
مركز البدرشين - الجيزة

١٩٨٦/٠١/١٥

٢٨٦.٠١.٥٢١.٢٢٦٩

KV7940474

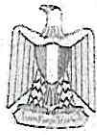
٠١٠١٣٤٩٦١٨٠



٢٠٢٤/١١ ٢٨٦.٠١.٥٢١.٢٢٦٩

ربة منزل

أنثى مسلمة أرملة



البطاقة سارية حتى ٢٠٣١/١١/٠٣



Patient's Name: Khadija Abdel Wahab Abdallah Heikal
Date: 2026-08-13
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Pathologically proven left breast IDC grade II, Luminal b Her 2 -ve molecular subtype, re-assessment study.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- No masses, speculations, asymmetry, distortion or suspicious microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs. No pathological infra-clavicular nodes.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- UOQ middle & anterior third focal asymmetry interconnected to a LIQ & central one; both surrounded with architectural distortion. No suspicious microcalcifications.
- Thickened skin, coarsened trabeculations and retracted nipple.
- Ultrasound demonstrated pathologically proven malignant process involving most of the breast of non-mass altered parenchyma mounting to two interconnected masses formation seen occupying the middle & anterior thirds of UOQ measuring about 7 x 4.5 x 4 cm and lower central, middle and anterior thirds measuring about 5 x 2.5 x 2 cm. the previously detailed process is seen reaching the nipple and the overlying skin at both sites.
- Thickened skin reaching 13 mm with coarsened trabecular and minimal subdermal edema.
- Retracted nipple.
- Multiple (more than three) pathological axillary LNs are noted at level 1, 2 & 3, the former two groups showed evidence of extra capsular rupture largest on level 1 measured 26 x 12 mm, on level 2 measured 18 x 13 mm, on level 3 measured 13 x 7mm, pathological multiple left supra-clavicular nodes are noted too largest measured 10 x 9 mm.
- Indeterminate solitary right supra-clavicular node is noted measuring 7 x 6 mm.

Conclusion:-

Right breast (BIRADS 1): Normal study.

Left breast (BIRADS 6): Pathologically proven Locally advanced malignant process with ipsilateral pathological level 1, 2, 3 axillary & supraclavicular lymph nodes with appreciated extra-capsular rupture at former two groups. Indeterminate right supraclavicular lymph node

Recommendation:-

For right supra-clavicular lymph node biopsy and histopathological correlation.

Best Regards,
DR. Amal Hegazi, M.Sc.

Name : Khadija Abdel Wahab Abdallah Heikal	Date : 05-09-2026 11:02
Branch : Nasrcity	Medical No. : 454674

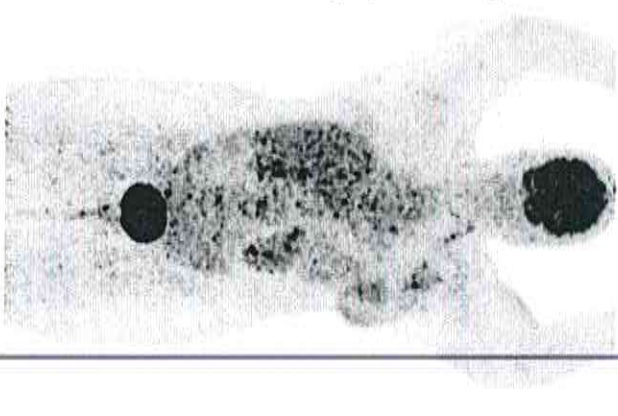
Whole Body F18- FDG PET/CT study

Clinical history:-

40 year old female patient , presenting with left breast mass referred for PET CT study
Type of the study :first time

rocedure:

- ✗ **Radiopharmaceutical:** 18F FDG, 3.2 mCi I.V.
- ✗ **Blood glucose level at time of 18F-FDG injection:** 199 mg/dL .
- ✗ **Time from 18F-FDG injection to scan:** 60 min
- ✗ **Technique :**MDCT diagnostic examination was taken for attenuation correction anatomic localization followed by PET images from the skull vault to the mid thigh were obtained.
- ✗ Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.
- ✗ **Patient's height:** 160 cm **patient's weight:**77 kg
- ✗ **SUV average of reference hepatic activity=**1.4 .
- ✗ **Measurements are correlated to body weight.**



PET/CT FINDINGS:-

Head and Neck:

- No metabolically active cervical LNs noted.
- The brain exhibited normal FDG bio-distribution
- Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- Normal thyroid uptake

Chest:

- The left breast is seen enlarged with diffuse parenchymal heterogeneity noted and a low grade metabolic active lesion seen implicating the LLOQ measures 22x27 mm with SUV max of 2.5
- Low grade metabolic active skin thickening reaching SUV max of 2.3 measures 10 mm
- Multiple metabolic active left axillary and left lower deep cervical / supra clavicular LNs are noted the most avid measures 14x18 mm with SUV max of 6
- Few scattered tiny non FDG avid non specific pulmonary nodules for close follow up

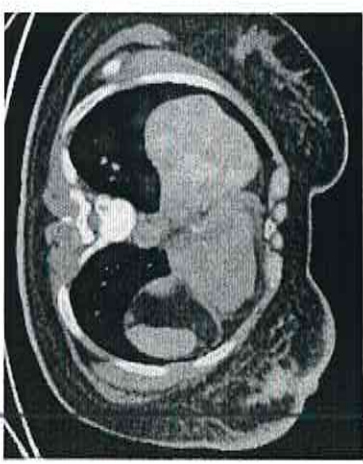


Name : Khadija Abdel Wahab Abdallah Heikal	Date : 05-09-2026 11:02
Branch : Nasrcity	Medical No. : 454674

- Normal FDG uptake is seen throughout the both lung lobes with no FDG avid lung nodules or masses detected.
- There are no pleural or pericardial abnormalities.
- Physiologic FDG uptake is noted in the myocardium.

Abdomen and Pelvis:

- No evidence of hepatic or splenic FDG avid focal lesions.
- No metabolically active pelvi-abdominal LNs noted.
- There are no FDG avid adrenal nodules.
- No evidence of FDG avid peritoneal nodules
- Physiologic FDG excretion is seen in the kidneys and bladder.



Musculoskeletal:

- Normal marrow activity is seen along the surveyed skeleton
- No sclerotic or lytic lesions

Conclusion:

- First PET/CT study reveled metabolic active left breast LOQ lesion associated with multiple metabolic active left axillary and left lower deep cervical and supra clavicular LNs noted for histopathological correlation

Thanks for referral;
Dr. NORA NABIL, MD
Lecturer of Radio-diagnosis,
ASUH

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9/13/26, 1:37 PM

code:09 – Name_A MDT FORM – Name_E MDT FORM

د. يوسف
روسي
رئيسة
قسم
السرطان

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أ. ب. ج.



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الجمهورية
مصر
العربية

09/13/2026

MDT FORM

Name:	كريمة عبدالرحمن عبدالمجيد	MRN:	11584
Age:	41 Y	ID:	28601052102269
Menopausal Sta	pre	Phone:*	01042767183
NRS SCORE:		Marital Status:	widow
BMI:		Family History:	-ve
Gender		Reason Of MDT:	new
CO Morbidity:	DM	ECOG Performance Status	0 1 2 3 4
Surgical History:*	umbilical hernioplasty		

Clinical Data:

R

L

BX: IDC 2

ER: 8/8 PR: 5/8 HER2: 2+ not a KI67: 15 %

History:

Sonomammography:

LT Locally advanced involving most of breast + skin thickened + Lt axillary
LNs + multiple LT supradlav. + rt indeterminate supradlav.

Biomarker/Molecular

Luminal A

Mets Workup:

PET CT lt bresat LOQ lesion + lt lower cervical LNs
RT supradlav. biopsy refused

Recommendations:

Clinical Staging:
T4b N 3c Mx

MDT Outcome & Plan

LOCALLY ADVANCED N3
PORT-CTH
ECHO
ACX4 TXT X3 US EVERY 2 CYCLES
LT MRM AFTER EVALUATION

Pathological Staging:

Q1 Right Side Q2 Q3 Q4 Q1 Left Side Q2 Q3 Q4

T N M



SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
2 محمد كركاش	1 محمد كركاش		

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